

Correspondence on “Incidence and predictors of hepatocellular carcinoma in patients with autoimmune hepatitis”

To the Editors:

We extend our sincere appreciation to our colleagues^{1,2} for their valuable comments and suggestions regarding our recent publication on the risk of hepatocellular carcinoma (HCC) development in patients with autoimmune hepatitis (AIH). We acknowledge the necessity for further evaluation of the impact of immunosuppressive therapy on HCC development to customize treatment strategies for at-risk groups. As noted by Dr Shan, the unexpected non-significant role of immunosuppressive treatment in HCC development in our study can be primarily attributed to the small number of HCC cases observed (24/1,428). We are aware that previous studies have indicated an association between immunosuppressants and primary liver tumors or all-tumors both in AIH and other autoimmune diseases.³ However, although oral corticosteroids have been associated with an increased risk of HCC development, with a duration-dependent feature,⁴ it is important to highlight that response to treatment has also been associated with reduced adverse outcomes in AIH,⁵ suggesting a delicate balance between the two risks.

We further agree with Pasta and colleagues¹ and strongly advocate for an adequate characterization of risk factors and comorbidities, such as alcohol consumption and metabolic components, in patients with AIH. Unfortunately, data on metabolic risk factors (*i.e.*, diabetes mellitus, hypertension, and dyslipidemia) have not been thoroughly investigated in this specific population and deserve further studies. Recognizing the need to focus future research on specific, at-risk groups of patients with AIH, the community is diligently working towards prospective data.

Despite the intrinsic limitations of a retrospective multicenter study, our main takeaway is that the risk of HCC in AIH is low, even after cirrhosis development, and that personalized screening strategies should be designed based on specific risk factors. Considering the cost-effectiveness of routine monitoring of patients with long-term disease and its impact on quality of life, we recommend tailoring management strategies. Surveillance should be prioritized in at-risk subgroups, such as

those diagnoses at an older age, those with established cirrhosis, or those with PSC-variant syndrome, as well as those with concomitant comorbidities for liver diseases including alcohol consumption. While current recommendations for HCC surveillance refer to people living with cirrhosis independently of the etiology, our data show that the majority of patients with AIH would not benefit from a 6-month ultrasound.

In conclusion, we thank our colleagues for their valuable insights and recommendations. We advocate for studies aiming to delve deeper into the complex facets of AIH, to assess the impact of immunosuppressive therapies and metabolic risk factors, and thereby gather more nuanced information. We further recognize the importance of prospective collaborative studies, and we are committed to continuing our efforts in this regard.

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Conflict of interest

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Authors' contributions

Both authors performed literature search, data interpretation, and writing of the manuscript. The final version was approved by all authors.

Supplementary data

Supplementary data to this article can be found online at <https://doi.org/10.1016/j.jhep.2024.05.019>.



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